Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

ACUTE PAIN

Ward Management of Patients Receiving Intrathecal Opioids

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Author(s): Dr Ian Johnson, Miss Sonya Jones
Reviewer: Departmental Guideline Group

Key Points

- Intrathecal opioids may provide effective pain relief for up to 24 hours post procedure.
- For otherwise reasonably healthy patients an intrathecal dose of diamorphine up to 1mg may be used.
- Post operative monitoring following IT diamorphine follows that similar to patients in whom a PCA is used.
- If patients require more frequent monitoring or have received more than 1mg of diamorphine then ward based care is not appropriate.
- IT morphine should not be used with inpatients returning to general surgical wards

Introduction:

Spinal anaesthetics are commonly used for operations on the lower part of the body. This may be with a local anaesthetic only or opioids may be used alone or in combination with a local anaesthetic. Patients receiving intrathecal opioids alone usually have a general anaesthetic as well.

The terms intrathecal opioids and spinal opioids are interchangeable.

Spinal / intrathecal opioids –

A small dose of intrathecal / spinal opioid (diamorphine) produces prolonged and effective analgesia. Monitoring is required to detect adverse effects. There is a small chance of late onset respiratory depression which will manifest as excessive drowsiness, reduced respiratory rate and pin point pupils. Early detection is imperative.

Limitation:

This guideline does not relate to epidural care, or other indwelling catheters or the administration of the other drugs via the intrathecal route, including low dose intra-thecal Fentanyl.

The guideline does not advise on the actual insertion of spinal opioids within theatres. It is the responsibility of the anaesthetist to ensure the patient is appropriate for the use of this technique.

This guideline is not suitable for intrathecal MORPHINE which has a higher risk of delayed respiratory depression.

Clinical Patient Group Criteria and Monitoring recommendations:

GROUP 1: Patients < 80 yrs of age who receive IT diamorphine dose ≤ 1mg

No major concern over other co-morbidity (anaesthetic assessment)

PLAN for Group 1: Suitable for ward based care

Observations: BP, HR, SaO_2, Resp rate every 15 mins for 2hrs

then: as before + sedation, nausea/ vomiting and pain scores

1 hourly for 4 hours

4 hourly up to 24 hours post op.

Also: May have PCA morphine / fentanyl for pain relief

May have oral opioids

Patent IV for 24 hours

GROUP 2: > 80 years

Or IT Diamorphine dose >1mg <2mg

Or Any patient with significant co-morbidity

Or Patients already on strong opioids (MST, fentanyl patch etc)

PLAN for Group 2: As for Group 1 except:

Monitor for **1 hourly for 12 hours** (after initial 15mins for 2 hours) then **4 hourly up to 24 hours post op.**

THIS LEVEL OF CARE MAY PRECLUDE WARD-BASED CARE OF THESE PATIENTS.

Transfer from recovery to the ward

Transfer to the ward will take place when the patient has been monitored closely for not less than half an hour and observations are stable.

Patients should be prescribed oral opioids OR PCA by the Anaesthetist post-operatively depending on the type of surgery.

Regular paracetamol and NSAID (if no contra-indication) should be prescribed and

should be administered regularly.

Two antiemetics should be prescribed – refer to local guidelines on management

of post-operative nausea and vomiting.

Patients may sit up slowly at any time following spinal anaesthesia/intrathecal

analgesia. There is no time limit that they have to remain lying flat, unless they

have a low BP, complain of headache, or for specific post-operative reasons

related to the operation.

Management of Side Effects or Complications:

Respiratory Depression:

If the respiratory rate is <8 per minute and the patient is not awake and not responding to voice use naloxone. Prepare naloxone 400 micrograms made up to 4 mL with sodium chloride 0.9%. Contact medical team to review patient. Administer naloxone 100 micrograms (i.e. 1 mL) every 1 minute until respiratory rate >10 and the patient is responding to voice.

The effect of naloxone may wear off in less than 1 hour and further doses may be

required.

If respiratory depression develops, avoid further opioid doses, remove PCA handset from patient's reach. Seek advice from the pain team/on-call anaesthetist if pain remains a problem.

Sedation

Sedation is the earliest sign of respiratory depression.

If the patient is not responding to voice, maintain airway if required, give oxygen

using a facemask with reservoir bag at a flow rate of 15 litres/minute and call

medical assistance. Prepare naloxone 400 micrograms made up to 4 mL with sodium chloride 0.9%. Administer naloxone 100 micrograms (i.e. 1 mL)

every 1 minute until respiratory rate >10 and the patient is responding to voice

Nausea and vomiting

Administer anti-emetic as prescribed.

Itching

Treatment options include

Chlorphenamine (caution, causes sedation)

For severe itching resistant to chlorphenamine consider

ondansetron 4mg stat IV (unlicensed use)

naloxone 20 microgram IV titrating up to 2 micrograms/kg IV

If necessary, contact on-call anaesthetist.